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Anat Path Reports

UUID:8892002E-D9B2-47FA-9540-A19E4A35B6C7
TCGA-95-A4VP-01A-PR

Redacted

* Final Report *

Document Type: Anat Path Reports
Document Date:
Document Status: Auth (Verified)
Document Title: Histology
Performed By:
Verified By:
Encounter info:

* Final Report *

ICD-O-3
adenocarcinoma, acinar 8550/3
Site: lung, lower lobe C34.3
jw 10/27/12

APRPT (Verified)

Patient Name:
MRN:
Location:
Client:
Submitting Phys:

DOB:
Gender: F

Acc #:
Pulmonary Big
Collected:
Received:
Reported:

Final Surgical Pathology Report

Final Pathologic Diagnosis

A. LUNG, PLEURAL DIAPHRAGM, BIOPSY, A1FS:
- **PAPILLARY MESOTHELIAL HYPERPLASIA.**
- **NO EVIDENCE OF ADENOCARCINOMA.**
- **SEE COMMENT.**

B. LUNG, PLEURA, BIOPSY, B1FS:
- **PAPILLARY MESOTHELIAL HYPERPLASIA.**
- **NO EVIDENCE OF ADENOCARCINOMA.**
- **SEE COMMENT.**

C. LYMPH NODE, LEFT LEVEL 9, EXCISION, C1FS:
- **ONE LYMPH NODE POSITIVE FOR METASTATIC ADENOCARCINOMA (1/1).**

D. LYMPH NODE, LEFT LEVEL 11, EXCISION:

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- **TWO OF FIVE LYMPH NODES POSITIVE FOR METASTATIC ADENOCARCINOMA (2/5).**

E. LUNG, LEFT LOWER LOBE, LOBECTOMY:
- **ADENOCARCINOMA, MODERATELY DIFFERENTIATED, ACINAR, 6.9 X 4.7 X 4.1 CM.**
- **TUMOR EXTENDS TO VISCERAL PLEURAL SURFACE.**
- **LYMPHANGITIC INVASION IDENTIFIED.**
- **BRONCHIAL AND VASCULAR RESECTION MARGINS ARE NEGATIVE FOR CARCINOMA.**
- **FOUR OF EIGHT PERIBRONCHIAL LYMPH NODES POSITIVE FOR METASTATIC ADENOCARCINOMA (4/8).**
- **SEE STAGING SUMMARY AND COMMENT.**

F. LYMPH NODES, LEFT 7, EXCISION:
- **TWO LYMPH NODES NEGATIVE FOR ADENOCARCINOMA (0/2).**

G. LYMPH NODES, LEFT 5, EXCISION:
- **THREE LYMPH NODES NEGATIVE FOR ADENOCARCINOMA (0/3).**

Comment
The tumor is diffusely positive for TTF-1, and focally positive for synaptophysin, confirming the diagnosis of adenocarcinoma.

Within the hyperplastic mesothelial lesions, immunohistochemical stains for TTF-1 are negative.

Synoptic Worksheet
E. Left lower lobe:

Specimen:	Lobe(s) of lung: left lower
Procedure:	Lobectomy
Specimen Integrity:	Intact
Specimen Laterality:	Left
Tumor Site:	Lower lobe
Tumor Focality:	Unifocal
Histologic Type:	Adenocarcinoma
Histologic Grade:	G2: Moderately differentiated
Tumor Size:	Greatest dimension: 6.9 cm Additional dimension: 4.7 cm Additional dimension: 4.1 cm
Visceral Pleura Invasion:	Present
Tumor Extension:	Not applicable
Bronchial Margin:	Uninvolved by invasive carcinoma Involvement by squamous cell carcinoma in situ (CIS) not applicable

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Vascular Margin:	Uninvolved by invasive carcinoma
Parenchymal Margin:	Not applicable
Parietal Pleural Margin:	Not applicable
Chest Wall Margin:	Not applicable
Other Attached Tissue Margin:	Not applicable
All Margins Uninvolved By Invasive Carcinoma:	
	Distance of invasive carcinoma from closest margin: 16 mm
	Margin closest to invasive carcinoma:: bronchial
Treatment Effect:	Greater than 10% residual viable tumor
Lymph-Vascular Invasion:	Present
TNM Descriptors:	y (post-treatment)
Primary Tumor (pT):	pT2b: Tumor greater than 5 cm, but 7 cm or less in greatest dimension
Regional Lymph Nodes (pN):	pN2: Metastasis in ipsilateral mediastinal and/or subcarinal lymph node(s)
	Nodes examined: 19
	Nodes involved: 7
Distant Metastasis (pM):	Not applicable

Clinical History
VATS. Lung cancer (left lower lobe). Left VATS, possible thoracotomy with left lower lobectomy.

Specimen(s) Received
A: Pleural diaphragm biopsy
B: Pleural biopsy
C: Left level 9 lymph node
D: Left level 11 lymph node
E: Left lower lobe
F: Left 5 lymph node
G: Lymph node

Gross Description
Specimen A is received fresh for intraoperative consultation labeled with the patient's name, medical record number and as "pleura diaphragm." It consists of multiple portions of soft tissue measuring 0.7 x 0.6 x 0.2 cm in aggregate. The specimen is submitted entirely for frozen section diagnosis, then transferred to cassette "A1FS." (Dictated by

Specimen B is received fresh for intraoperative consultation labeled with the patient's name, medical record number and as "pleural biopsy." It consists of a portion of soft tissue measuring 1.0 x 0.6 x 0.1 cm. The specimen is submitted entirely for frozen section diagnosis, then transferred to cassette "B1FS." (Dictated by

Specimen C is received fresh for intraoperative consultation, labeled with the patient's name, medical record number and as "left level 9 lymph node." It consists of one lymph node measuring 2.0 x 2.0 x 0.8 cm. The specimen is bisected. Half of it is submitted for frozen section diagnosis, then transferred to cassette "C1FS." The other half is submitted for permanent section in cassette "C2." (Dictated by

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Specimen D is received in formalin, labeled with the patient's name and medical record number and as "left level 11 lymph node." The specimen consists of multiple portions of fibroadipose tissue measuring 2.0 x 1.0 x 0.3 cm in aggregate. The specimen is submitted entirely in cassette "D1." (Dictated by

Specimen E is received fresh in a container labeled with the patient's name and medical record number and "left lower lobe." The specimen consists of a portion of lung measuring 13.6 x 10.4 x 8.6 cm. The pleural surface is irregular with multiple areas of puckering, the largest measuring 1.7 x 1.4 cm. The tumor is grossly identified at the pleural surface. Five lymph nodes are grossly identified ranging in size from 1.0 cm to 2.1 cm. The tumor measures 6.9 x 4.7 x 4.1 cm. The cut surface of the tumor is tan-white and soft with focal areas of necrosis and hemorrhage. It is 1.6 cm from the bronchial margin and 2.1 cm from the vascular margin. No other lesions are noted within the lung parenchyma. The specimen is inked as follows: pleural surface, black; Sections are submitted as follows. (Dictated by

Specimen F is received in formalin, labeled with the patient's name, medical record number and as "level 7 lymph node." It consists of two portions of fibroadipose tissue measuring 1.4 x 1.0 x 0.3 cm in aggregate. The specimen is submitted entirely in cassette "F1." (Dictated by

Specimen G is received in formalin, labeled with the patient's name, medical record number and as "level 5 lymph node." It consists of multiple portions of fibroadipose tissue measuring 1.5 x 1.5 x 0.5 cm in aggregate. The specimen is submitted entirely in cassette "G1." (Dictated by

CASSETTE SUMMARY:

E1:	Bronchial margin
E2:	Vascular margin
E3:	One lymph node, bisected
E4:	One lymph node, bisected
E5:	Two whole lymph nodes, one bisected (inked blue)
E6-8:	Representative sections of tumor in relation to puckered pleural surface
E9:	Normal lung parenchyma
E10:	One lymph node, whole
E11-13:	Additional sections of tumor

Intraoperative Consult Diagnosis

A1FS: PLEURA, DIAPHRAGM (FROZEN SECTION): PAPILLARY MESOTHELIAL PROLIFERATION.
Frozen section results were communicated to the surgical team and were repeated back by
specimen was received at

B1FS: PLEURA, BIOPSY (FROZEN SECTION): PAPILLARY MESOTHELIAL PROLIFERATION MORE COMPLEX THAN PREVIOUS BIOPSY.
Frozen section results were communicated to the surgical team and were repeated back by

and the specimen was received at

C1FS: LEFT LEVEL 9 LYMPH NODE (FROZEN SECTION): EPITHELIAL MALIGNANCY IN LYMPH NODE, PENDING FURTHER IMMUNOHISTOCHEMICAL CHARACTERIZATION.
Frozen section results were communicated to the surgical team and were repeated back by

and the specimen was received at

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Microscopic Description
Microscopic examination performed.

Printed by:
Printed on:

	Yes	No
...ancy		
...ate Discrepancy		✓
...ncy		✓
...atory		✓
...Primary Noted		✓

QUALIFIED DISQUALIFIED
RB Date Reviewed 10/27/10
10/27/10